Burke’s Birth & Wellness Mobile Clinic

Bringing Mobile Maternal Care to Maternal Care Deserts

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December 9, 2025

BSHES526

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Project Introduction

Maternal and infant morbidity and mortality are significant issues in the United States, with 23.8 maternal deaths per 100,000 live births in 2020 and an infant mortality rate of 5.6 deaths per 1,000 live births in 2022, according to the CDC (Taylor, 2022). Infant mortality rates are also disproportionately high in the U.S. South (Hirai et al., 2014). An overwhelming number of these deaths could have been avoided with access to maternal care (Souza et al., 2024; Albarqi, 2025). This health concern is especially pertinent in Georgia, where in 2021, 85% of pregnancy-related deaths were considered preventable and 62% of pregnancy-related deaths were insured by Medicaid (Georgia Department of Public Health, 2025).

Burke County, Georgia, a rural maternal care desert, lacks obstetrics and gynecology (OB-GYN) offices (March of Dimes, 2024). Access disparities are worse for uninsured or Medicaid-covered individuals (Bellerose et al., 2022). Burke's Birth &Wellness Mobile Clinic, or Burke's Mobile Clinic, addresses this gap by offering maternal care to uninsured or publicly insured birthing-age people, throughout the perinatal period, defined in this program as conception up to three months postpartum (NHS, n.d.). Services include prenatal and perinatal education, peer connections, and access to OB-GYN services. This mobile clinic would also connect expectant mothers with a clinician, such as an OB-GYN, nurse practitioner (NP), or doula, to facilitate their birth. Burke's mobile clinic also provides infant care essentials like formula, diapers, wipes, and toys, requiring engagement with educational resources to receive these materials.

With the knowledge that education, healthy behavior, and access to care can prevent maternal and infant morbidity and mortality, Burkes Mobile Clinic hopes to bridge the gap in care that exists in Burke County in order to create a safe and helpful environment for future parents to learn, seek care, and ultimately have a healthy birth and child.

Community Description

Burke County, Georgia, is a large and mostly rural county, about 300 miles southeast of Atlanta (US County Maps, 2024). It has a relatively small population of 24,427 people. However, the county is the second largest in the state in terms of land area (*Burke County by the Numbers | Burke County Chamber of Commerce*, 2019).

Demographics, Income, and Insurance

Burke County's median age is 38, with a nearly even gender split (marginally more females). Burke County is predominantly Black (45%) and White (48%), with a small Hispanic (3%), Asian (1%), and Native American (0.1%) population (Census profile: Burke County, GA, 2023). 54% of the county's residents are employed with a median household income is about $50,000. 19.2% of the population has an income below the federal poverty line. Additionally, about 12% of the population has no insurance (Census profile: Burke County, GA, 2023).

Birthing Individuals

There are around 5,500 individuals of birthing age (22.5% of population) defined by the census as women from ages 15-50. Average family size 3.42, with 260 births in the last 12 months. Despite this, there are no OB-GYN practices located anywhere in the county (Census profile: Burke County, GA, 2023). Nationally, people who give birth in rural areas must travel longer distances over greater periods of time to seek maternal care. In Georgia, individuals travel an average of 13.5 miles and 19.9 minutes to get to the nearest birthing hospital. This number is even higher in rural Georgia counties, like Burke County, where 61.0% of women live over 30 minutes from a birthing hospital, significantly higher than the 13.0% of women living in urban areas (Fontenot et al., 2023). This disparity contributes to inequities in maternal health outcomes.

Transportation Available

Many households have transportation barriers, and public transportation is not easily accessible in this county. The CDC estimates that 14% of adults have transportation barriers (2024), and the American Community Survey estimates that 8% of occupied households have no vehicle available for use (US Census Bureau, 2023).

Burke Transit does have a rural transportation service available upon request on most weekdays. This service provides trips to medical appointments, work, and school for a round-trip fee of $6-10. It also offers trips to the neighboring county for additional payment ("Welcome to Burke County," n.d.). Despite this service, the county scores a 0 out of 10 on the U.S. Department of Transportation's transit tool, AllTransit. This is the lowest possible ranking, reflecting ineffective and inequitable public transit service available (AllTransit, n.d.).

Rural Maternal Health Disparities

These transportation challenges do not exist in isolation; they intersect with broader social and geographic factors that worsen reproductive health disparities. Georgia continues to have one of the highest maternal mortality rates in the country, with rural residents facing risks up to two times as high as their urban counterparts. The disparities are particularly stark for Black women in rural Georgia, who face these rates at twice the level of rural White women and roughly 30% higher than Black women in urban parts of the state (Robinson, 2025).

The socio-ecological framework illustrates how racism underlies the social determinants of health (SDOH) and drives inequitable reproductive health outcomes for Black women (Prather et al., 2016). This occurs across multiple levels, including individual factors such as elevated stress, neighborhood factors such as unequal healthcare access, and societal factors such as institutionalized racism (Prather et al., 2016**).** Given that Black residents comprise 45% of Burke County's population and people of color constitute just over half, addressing structural racism must be embedded throughout the program curriculum.

Maternal Care Desert

Factors like rurality, transportation barriers, and racism can impede access to maternal care, but they do not automatically qualify an area as a maternal care desert. Burke County is a maternal care desert because it has no hospitals and birth centers offering obstetric care, no obstetric providers, and a proportion of adult women without health insurance. This has serious implications for county residents, because maternal care deserts are associated with an increased risk of poor health outcomes, including poor maternal mental health, maternal morbidity and mortality, and infant pre-term birth (Gleeson et al., 2025; Fontenot et al. 2023).

This program seeks to understand and address the overlapping factors that increase the risk of poor reproductive outcomes in Burke County. Through text message-based surveys and key informant interviews, we seek to identify the strengths and barriers that Burke County residents and community partners report as influencing reduced access to maternal care. Because of its rural setting, limited public transit, demographic makeup, and general lack of obstetric care, Burke County requires close attention and strategic intervention to address the public health problem of access to maternal care.

Needs Assessment

Secondary Data Collection

To most effectively target our intervention to our specific population, a needs assessment will be conducted. For secondary data collection, there are several national resources like the

National Vital Statistics System, Pregnancy Mortality Surveillance System, and Pregnancy Risk Assessment Monitoring System that document factors like population fertility, birth outcomes, maternal mortality, and pregnancy risk at the county level. They provide descriptive statistics on maternal health in Burke County and quantify the need for care or intervention.

In addition to these national resources, a data-sharing agreement with the Burke County Health Department would provide specific and up-to-date metrics on maternal health outcomes. This real-time and local information would prove invaluable to establishing the priorities and direction of the present program plan. Community partners can provide both empirical and anecdotal data concerning maternal healthcare in Burke County. Community organizations, like Community Health Care Systems, Inc., likely compile data on birth outcomes, service usage, and community demographics as a routine part of operations.

Primary Data Collection

It is also essential to gather comprehensive information from birthing individuals in the community and local organizations about prevailing attitudes and beliefs regarding maternal care. We will use surveys and key informant interviews (KII) to identify community strengths and barriers, including available resources, perceived support, and perceived obstacles to accessing maternal care.

Individual surveys will be collected through a text message-based platform and be sent to birthing people of reproductive age in Burke County. These individuals will be located through local public health programs, such as Women, Infant, and Children centers (WIC), local hospitals, health departments, and other community programs. Brief open-ended questions, such as, “what are some reasons you did not see the doctor during your pregnancy?” will be asked to capture attitudes about maternal care, as well as perceived barriers and supports specific to the local community. We aim to get at least 250 responses, representing ~5% of the population of birthing individuals in Burke County.

Additionally, KII, will be conducted with local organizations, such as hospitals and nonprofits, to obtain detailed information about what resources are being used, who community leaders and changemakers are, as well as what gaps the organizations have noted in maternal health care. A semi-structured interview guide will be used with questions such as "What gaps have you noticed in maternal health resources?" We aim to conduct at least 4 KII to align with recommendations for community readiness assessments (Kostadinov et al., 2015).

The primary data collected may complement, challenge, or contextualize the secondary data available for Burke County. All together, these various sources can inform an effective and comprehensive program tailored to the specific needs of Burke County residents.

Analysis of the Problem

The United States has a maternal mortality rate of approximately 23.8 deaths per 100,000 live births (Taylor, 2022) and an infant mortality rate of 5.6 deaths per 1,000 live births (CDC, 2024). Of these deaths, an overwhelming number were due to avoidable causes. These statistics highlight how maternal care is central to the health of the mother and the outcome of the baby before, during, and after birth (Leung, 2023). As mentioned, these mortality rates are even higher among minority populations (Taylor, 2022). Many of these outcomes are driven by SDOH. Structural racism, gender identity, sexual orientation, and socioeconomic status (SES) profoundly shape these determinants, contributing to disparities in maternal health (Dagher & Linares, 2022).

In Georgia, maternal mortality remains a critical public health concern, with a rate of 37.9 deaths per 100,000 live births from 2019 to 2023 (Healthy Mothers, Healthy Babies Coalition of Georgia, 2025). This alarming rate ranks the state seventh highest nationwide, making it imperative to prioritize solutions (Healthy Mothers, Healthy Babies Coalition of Georgia, 2025).

Georgia's outcomes for infant mortality are also concerning with approximately 7 infant deaths per 1,000 births (Georgia Maternal & Infant Health Data, 2025). This number ranks fifth highest in infant mortality when compared to other states (CDC: Infant mortality, 2025). In 2023, 10.2% of live births in Georgia resulted in low-birth-weight infants (March of Dimes, 2023). Statistically, Georgia consistently has poorer outcomes in low infant birth weight, maternal mortality, and infant mortality when compared to the rest of the country (March of Dimes, 2023), highlighting the need for more targeted interventions to address maternal care.

Inadequate prenatal care contributes significantly to this problem (Partridge et al., 2012). According to the March of Dimes (2023), Georgia sees an average of 2,400 babies born weekly, yet only 1,700 mothers report receiving adequate prenatal care. By offering education and adequate care during and after pregnancy, Burke Mobile Clinic aims to bridge this gap.

These issues are critical in Burke County. The March of Dimes rates Burke Couty as 89.2 on the Maternal Vulnerability Index (March of Dimes 2023), indicating that there is a high risk of adverse health outcomes for birthing people in this county. Barriers including poor transportation, high travel distances, inadequate insurance, and limited resources contribute to this issue (March of Dimes, 2024; Fontenot et al., 2024). While there is a WIC center, approval for coverage by this program is a long process that requires many approvals and signatures, which may be a barrier for many women to complete in a timely manner (Figueroa et al., 2025).

Additionally, some people may not be eligible for this and be left uninsured or without coverage for the specific services they need.

In the long term, preventable maternal illness and death create a poorer quality of life in maternal care deserts. Research in other states indicates that pregnancy-associated death was linked with infant death and worse health of surviving children in the first year of life (Declercq et al., 2025). These surviving infants had a higher risk of hospitalization within the first year of life. SDOH, such as racism or poor quality of healthcare, can also lead to accelerating biological aging, which in turn is associated with lower infant birth weight (Dagher & Linares, 2022). Infants who are born with low birth weight or with pregnancy complications are more likely to have neurodevelopmental conditions and long-term health conditions (Blencowe et al., 2019). Mothers who experience adverse birth outcomes are more likely to suffer long-term health consequences and mental health conditions such as anxiety and depression (Bodunde et al., 2024). These long-term impacts on quality of life demonstrate how addressing these maternal health disparities is a pressing public health issue.

PRECEDE Analysis

A PRECEDE Analysis (Gielen et al., 2008) of access to maternal care was conducted by the team and yielded the following behavioral and environmental determinants of the problem: See Figure 1 in **Appendix A** for more details.

Behavioral Factors

Teenage pregnancy: According to 2022 data collected by the Georgia Campaign for Adolescent Power and Potential, the teen pregnancy rate in 13.7 per 1,000, which is greater than Georgia's overall average of 9.9 per 1,000 (*Burke - GCAPP*, 2023). The CDC's guidelines for pregnancy planning include talking to a healthcare provider, getting enough folic acid, ceasing alcohol consumption, avoiding smoking and other toxic substances, as well as maintaining a healthy weight (CDC, 2024). Attending all recommended prenatal appointments impacts health outcomes as well (Khan et al., 2019).

Unfortunately, pregnant adolescents often face barriers to accessing adequate prenatal care, resulting in delayed or insufficient care (Gardner et al., 2023). This limited access increases the risk of adverse infant outcomes, such as preterm birth and low birth weight, as well as poor maternal outcomes, including higher rates of maternal mortality (Diabelková et al., 2023; Nam et al., 2022). Furthermore, these adolescents are more likely to engage in unhealthy behaviors, such as smoking, which can exacerbate these risks (Diabelková et al., 2023).

The use of contraceptives is an additional behavioral factor that influences family planning. Women in rural areas in the US are less likely to utilize contraceptives, partially due to lack of women's health providers. They are also less likely to engage in care-seeking behaviors (*Health Disparities in Rural Women*, 2024). Care-seeking with healthcare providers can be defined as any actions an individual with a perceived health problem takes to remedy the problem (Oberoi et al., 2016). Testing for pregnancy would also be considered care-seeking, which is less likely to occur with limited providers. Unplanned or late awareness of pregnancy can lead to late initiation of prenatal care, resulting in adverse outcomes including low maternal weight gain and higher risk of low-birth-weight infants (Haeri et al., 2022).

Environmental Factors

Burke County is the second largest county in Georgia by total area and is mostly rural (United States Census Bureau, 2023). This geography also impacts the population's ability to access essential health needs for pregnant individuals such as healthy food and places for physical activity. Spaces for physical activity are necessary to enable people to implement doctors' recommendations, such as getting exercise and maintaining a healthy weight.

Social (Interpersonal) Factors

Studies have found that instances of discrimination from providers are higher for uninsured and publicly insured individuals (Han et al., 2015). Stigma from the community against people who receive reproductive healthcare can also impact utilization, as certain religious groups may see family planning of any kind as abortion seeking (Fiacre Bazié et al., 2023). Lacking a social circle that prioritizes healthcare and partner relationship quality can hinder maternal care-seeking and worsen health outcomes (Stapleton et al., 2012).

Organizational Factors

Clinic appointments are often scheduled far in advance and rescheduling them can result in even longer waits for patients. A 2022 survey reported an average national wait time of 31.4 days for individuals seeking OB/GYN services (*Appointment Wait Times Increase across Specialties, but Not in Family Medicine*, 2022). US rural residents face a 39% higher patient burden for primary care access compared to urban residents (meaning a higher patient-to-physician ratio) and it is likely similar discrepancies exist with specialists like OB/GYNs (*States with the Biggest Discrepancies between Urban and Rural Healthcare Access*, 2025). Reduced access to OB/GYNs may lead to longer wait times for appointments and less flexibility when it comes to cancellations and rescheduling. Specialty clinics and hospitals often require further travel distances for rural residents, further exacerbating access inequities (Cyr et al., 2019).

Policy Factors

Lack of insurance coverage will influence care-seeking behaviors. According to data from the U.S. Census Bureau, 19.2% of persons in Burke County live below the poverty line, and 11.4% do not have health care coverage (United States Census Bureau, 2023). Of those who do have coverage, 62.7% have private health insurance, which can result in having large copays if the policy is very limited (United States Census Bureau, 2023).

Research shows that broadening public health insurance eligibility through Medicaid significantly increases low-income women's use of reproductive health services and strengthens access to preconception care (Margerison et al., 2020). Georgia's decision not to adopt Medicaid expansion under the Affordable Care Act has left a large portion of its reproductive-age population without affordable coverage. Women of childbearing age residing in states that declined expansion are two times more likely to lack health insurance compared with their counterparts in expansion states (Clark et al., 2021). Furthermore, Medicaid expansion has been consistently linked to reductions in maternal and infant mortality, with the most pronounced improvements observed among Black women and their infants (Clark et al., 2021).

Georgia has attempted to address part of this coverage gap through its Planning for Healthy Babies (P4HB) program, a Medicaid initiative that provides family planning services, prenatal vitamins, certain preventive screenings, and care coordination for eligible low-income women. Although P4HB represents a vital source of reproductive health coverage for individuals who would otherwise be uninsured, for most women, its scope is limited to family planning and select inter-pregnancy services (Centers for Medicare & Medicaid Services, 2021). The program does not offer comprehensive preventive services available under traditional Medicaid expansion. Therefore, while P4HB has demonstrated measurable benefits for enrollees, it likely cannot replicate the broader health gains associated with full Medicaid expansion for low-income adults (Centers for Medicare & Medicaid Services, 2021, 2024).

In addition to restricted insurance coverage, Georgia's post-Roe policies further limit reproductive care by enforcing a six-week abortion ban. This ban takes effect before most individuals realize they are pregnant and creates a significant barrier to abortion care for those who need it (ACLU, 2024). Research demonstrates that restrictive abortion laws are associated with adverse maternal and infant health outcomes, including increased infant and maternal mortality. These negative effects disproportionately affect underserved populations (Gemmill et al., 2025; Vilda et al., 2021).

Physical/Community Factors

In Burke County, 8.1% of households lack access to a vehicle, a factor that significantly compounds barriers to health care in a rural area where providers are often located many miles away (United States Census Bureau, 2023). As previously noted, the county is classified as a maternity care desert, with no labor-and-delivery services and very few physicians' offices available to residents (March of Dimes, 2023). Furthering these challenges, Burke County has only one WIC clinic to serve its entire population. This limited infrastructure is especially concerning given the county's elevated poverty rates and the evidence that non-participation in WIC is associated with increased risks of low birthweight, preterm birth, and infant mortality (Venkataramani et al., 2022).

These previously described environmental issues directly inform the behavioral and environmental determinants that this program will target low rates of prenatal care-seeking, the absence of community health workers (CHWs) to support families, inadequate transportation, a severe shortage of providers, and widespread lack of insurance coverage. Addressing these barriers together is essential to improving maternal and infant health outcomes in the county.

Rationale for the Environmental and Behavioral Determinants to Be Targeted By the Program

There is evidence supporting the rationale that implementing a mobile clinic model focused on maternal health care in Burke County will increase the utilization of these services among our population. Leibowitz et al. (2021) found that when barriers to health care access are removed, patients who visit mobile health clinics report increased satisfaction and express a high likelihood of using those services again. An analysis of the 2007 to 2017 Mobile Health Map found that mobile clinics most often serve women, people of racial/ethnic minorities, and the uninsured and publicly insured, demonstrating that these clinics serve some of the most vulnerable populations in a community (Malone et al., 2020).

While these clinics typically rely on philanthropy and government funding, this financial support allows these clinics to provide a variety of preventive health services to uninsured and underinsured patients at low or no cost (Malone et al., 2020). This would help alleviate the issue of expensive copays, a very real concern for those who are underinsured. Mobile clinics have been shown to make healthcare more accessible to people living in rural communities where they might live too far away from care, thus tackling the barriers of a lack of transportation and accessibility of doctors' offices (Iqbal et al., 2022). Burke County is a mostly rural county, so a clinic that can move closer to patients across the county would be a major asset.

Personal Determinants

Several personal determinants influence access to maternal care, including lack of knowledge and awareness, attitudes and beliefs toward perinatal care (the period during pregnancy and after birth), as well as health literacy and related skills.

An individual's lack of knowledge about maternal care, coupled with limited awareness of their pregnancy, directly impedes access to appropriate services. Birthing individuals from low-SES backgrounds often report unawareness of the importance of prenatal care visits, particularly when they feel well. This is especially pertinent in families with intergenerational poverty, who may have had several family members forgo prenatal care (Bellerose et al., 2022; Partridge et al., 2012). Those with a lack of knowledge about pregnancy may especially be unlikely to seek care when pregnancies are unplanned, as they may be unaware of signs and symptoms to alert them to seek care (Partridge et al., 2012; Meyer et al., 2016).

Attitudes and beliefs toward maternal care also play a critical role in determining the likelihood of seeking services. An individual's belief that the services they receive will be effective significantly influences their decision to seek care (Partridge et al., 2012). This is particularly relevant, as birthing individuals often report that poor communication, limited provider choice, and inconsistent care reduce their trust in the quality of services, deterring engagement (Meyer et al., 2016).

Motivation to engage in prenatal care can also be a challenge. Many low-SES women in rural communities face conflicting priorities due to financial strain, including hunger, lack of stable housing, lack of childcare, and demanding work schedules (Bellerose et al., 2022). This makes motivation to prioritize maternal care challenging. This is highly influenced by SES impacting access to financial, physical, and social resources, impacting the ability to access care and follow through on medical recommendations (Partridge et al., 2012).

Cultural, religious, and political beliefs surrounding maternal care further shape care-seeking behaviors. This is especially salient in regions with conservative religious and political views, where certain decisions about maternal care may lack support (Meyer et al., 2016). This may be exacerbated when a teenage or unwanted pregnancy comes into play, as an unwanted pregnancy may lead to denial of the situation and hesitancy to seek services (Bellerose et al., 2022). In addition, individuals may also fear shame by social networks due to this belief or decision (Bellerose et al., 2022).

Furthermore, these cultural and social pressures often intersect with perceived discrimination and stigma, impacting the decision to seek care. Individuals have cited experiences of stigma related to age, race, substance use, income level, drug use, undocumented status, HIV status, and insurance impacting participation in care (Bellerose et al., 2022; Meyer et al., 2016). These factors also connect to self-efficacy, as lack of insurance or reliance on public options can evoke feelings of discrimination. For example, individuals with Medicaid report longer wait times and restricted service days, leading to the feeling that they do not deserve equal access to care as those with insurance. In addition, fears of legal repercussions, whether due to immigration status or substance use, can result in care avoidance (Bellerose et al., 2022).

Beyond these attitudinal factors, difficulties with health literacy skills further impede birthing individuals' engagement with maternal care. Low-SES individuals are more prone to lower health literacy levels, which correlate with reduced health-seeking behaviors (Partridge et al., 2012; Vila-Candel et al., 2020). This affects their ability to navigate the healthcare system, as well as effectively implement recommendations from physicians, limiting the impact of care.

Determinants for Agents of Change

Improving outcomes for birthing persons requires addressing key agents of change, such as partners and other social supports, community leaders and stakeholders, local governments and nonprofit organizations, as well as local and national policies. Regarding determinants for agents of change, lack of social networks that have knowledge and skills related to care seeking can limit the support available for low-SES rural women in maternal care (Bellerose et al., 2022).

This is very important given that rural women cite older family members and other interpersonal connections as common sources of health information (Clark et al., 2022; Simmons et al., 2015). Also, lack of government prioritization of maternal health contributes to ongoing gaps in rural areas (March of Dimes, 2024). Lack of knowledge of health care providers regarding this issue and beliefs related to knowledge and skills of low-SES rural women in terms of maternal care can perpetuate barriers in service delivery (Morton et al., 2015).

The personal determinants of this intervention targets include lack of knowledge related to maternal care, attitudes and beliefs towards maternal care, and self-efficacy related to being able to address these needs. Several studies have shown that attitudes towards pregnancy and childbirth, as well as self-efficacy, can be addressed through educational interventions (Athinaidou et al., 2024; Timmermans et al., 2019). This demonstrates that these determinants are both impactful and changeable, making them effective targets for addressing this issue.

Interventions to Improve Uptake of Maternal Care in the United States

There have been numerous interventions that aim to address this issue, spanning a variety of settings including OB-GYN, primary care clinics, inpatient units, community sites, mobile care units, and at patients' homes (Gordon et al., 2025). Interventions included telehealth, social support through home visits and groups, workforce training and diversification, case management involvement and educational interventions (Gordon et al., 2025). Common goals for these interventions included connecting to the continuum of care, increasing frequency of clinical care visits, developing social support, providing education, and improving self-efficacy (Gordon et al., 2025). Notably, few interventions prioritized connecting women to healthcare during early pregnancy.

One intervention employed a mobile van to enhance early access to prenatal care in Palo Alto, California. The study found that women receiving prenatal care via the Women's Health Van accessed care earlier, with quality comparable to that provided at other community health clinics (Edgerleyet al., 2006). Research on primary care mobile health clinics have found that they have primarily been used by vulnerable populations, and have positive effects on decreasing emergency room use, decreasing length of hospital stays, and promoting chronic disease management (Coaston et al., 2022). While these outcomes are not directly tied to our population, the skills required for managing maternal care and service utilization align closely with those needed for medical and chronic disease management.

The most effective programs succeeded by making the community a central partner through using local resources to address diverse SDOH needs, working within existing healthcare systems, and building on community strengths to improve outcomes (Gordon et al., 2025). Our program applies these principles by engaging community stakeholders (including local mothers, fathers, grandparents, and partners of birthing individuals) as well as healthcare providers and leaders in the development process to guide the intervention.

Socio-ecological Approach to Program

Although barriers to maternal care span the socio-ecological model (Bellerose et al., 2022), the most frequently cited obstacles and interventions align with community and structural levels (Bellerose et al., 2022; CDC, 2022; Gordon et al., 2025). These include challenges with Medicaid coverage, provider shortages, and limited transportation (Gordon et al., 2025). Transportation, as a critical barrier, operates at the community level, shaped by infrastructure like public transit availability and distance to healthcare facilities. Our intervention addresses these core issues by eliminating coverage requirements and enhancing provider access through community-space services, thereby reducing structural barriers.

The Health Impact Pyramid, a framework for prioritizing public health interventions based on their potential population impact, underpins the theoretical bases of our intervention (Frieden, 2010). While our mobile health clinic incorporates individual education and clinical services, its primary impact derives from reducing structural barriers. By doing so, we are "changing the context" to integrate prenatal care into the community experience, making access a part of everyday life. This multi-level approach ensures our intervention maximizes impact by addressing systemic barriers to care while reducing individual-level effort (Frieden, 2010).

Program Goals and Objectives

The major goals and objectives of this program as well as their rationale are as follows:

Program goal 1: Increase physical access to maternal health services.

- **Process Objective 1:** Within the first six months of program implementation, the clinic will recruit and train two Burke County residents to become community-based doulas for Burke's Mobile Clinic.
- **Process Objective 2:** For each month of program delivery, both community doulas will schedule at least one visit with 4-6 program participants per month.
- **Outcome Objective 1:** At least 80% of pregnant people in the program will access maternal health services at least once a month during the duration of their participation in the program.

Rationale:

March of Dimes found that people giving birth in maternal care deserts receive less prenatal care and face a 13% higher risk of preterm birth. Residents of these deserts are uninsured at twice the rate of those in areas with full access (Stoneburner et al., 2024). This is especially alarming because fertility rates in rural counties and maternity care deserts are higher than those in urban areas (Stoneburner et al., 2024). As fertility rises, so does the population at risk. Our mobile clinic will address proximity, access, and affordability. By increasing physical access to services, the clinic aims to mitigate adverse outcomes linked to maternal care deserts.

Maternal care deserts also suffer from limited providers. Utilizing community-based doulas can effectively bridge this gap. Doula guidance in perinatal care is linked to positive outcomes, including fewer cesarean sections, premature deliveries, and shorter labor (Sobczak, 2023). This program will recruit and train two Burke County residents as community-based doulas to build trust and raise awareness of mobile resources (Kim et al., 2016). This early objective allows doulas and maternal care providers to collaborate on services and education for participants throughout the program. The International Doula Institute states that it is standard practice for one doula to take up to 6 clients per month (Bancoff, 2019). Having two part-time doulas employed by the program will allow for greater participant reach while also remaining within the program's budget. This program will follow the example of Building Perinatal Support Professionals (BPSP) program, which is facilitated by health Mothers, Healthy Babies Coalition of Georgia with a six-month timeframe. The six-month time frame will allow for completion of the full-spectrum doula training course as well as completion of the doula certification process (Healthy Mothers, Healthy Babies, 2022).

In addition, frequent access is important, as research shows maternal care during pregnancy reduces risks like preterm birth and chances of high-risk pregnancy (Vintzileos, 2002), with higher frequencies linked to better outcomes (Beyuo, 2023). This is why our program ensures contact with providers at least once per month. Further, 80% was chosen, because it is the Healthy People 2030 goal for percentage of women receiving early and adequate prenatal care (Healthy People 2030, 2022).

<u>Program goal 2</u>: Increase knowledge of positive maternal health behaviors, practices, and resources.

- **Process Objective 1:** The clinic will develop and deliver 12 maternal health education workshops to program participants once a month for the duration of the program.
- **Outcome Objective 1:** By the end of the program, 80% of program participants will report improved knowledge of positive maternal health behaviors and resources (adapted pre-test/post-test scale).

Rationale:

Maternal health care deserts, like Burke County, contribute to a maternal health gap stemming from absent services, limited knowledge, and attitudes toward prenatal care (March of Dimes, 2023). A way to combat this is by increasing health care literacy among people of birthing age and pregnant people (US Department of Health and Human Services, 2020). We aim to educate pregnant people and their community on maternal care benefits and healthy pregnancy practices. We also will connect them to physical and virtual resources to increase awareness of available support.

Increasing knowledge of health behaviors and resources can improve individuals' self-competence (Zaman et al., 2025). Our objective to achieve this outcome for 80% of participants aligns with Healthy People 2030's goal of ensuring early and adequate prenatal care. Enhanced knowledge has been shown to promote health-seeking behaviors in other populations (Ramos, 2023); we theorize that it will yield similar benefits for pregnant individuals.

<u>Program goal 3</u>: Increase participation of social support networks in maternal health.

- **Outcome objective 1:** By the end of the first year, 60% of program participants will have a support person attend at least 50% of maternal health education workshops with them.
- **Outcome objective 2:** By the end of the first year, 60% of program participants will have a support person attend at least 50% of their prenatal care visits with them.

Rationale:

Social networks are the interconnected systems of relationships surrounding individuals (Heaney & Israel, 2008). A key function is providing support to those within the network. Studies show that social support promotes better health behaviors, such as adequate prenatal visits and avoiding alcohol and substances, while fostering positive pregnancy experiences for birthing people (Al-Mutawtah et al., 2023; Schaffer & Lia-Hoagberg, 1997).

Birthing people who receive social support have an increased likelihood of pursuing pregnancy-related health information than those who lack social support (Guillory et al., 2014; Collins et al., 1993). Additionally, having social support is positively associated with birthing people initiating healthy behaviors such as participating in prenatal education (Schaffer & Lia-Hoagberg, 1997). This demonstrates the importance of having partners, family members, or friends of birthing people present during prenatal care visits and health education classes.

There is currently no formal guidance for the number of prenatal visits or educational classes that support people should attend. However, studies show it is feasible to expect support persons to attend at least 50% of education classes and prenatal visits (Mirsa et al., 2024; Salvesen von Essen et al., 2021). Similarly, there is no formal guidance on what proportion of birthing persons must have their support network involved in their care for a program to be effective; however, one study suggests that approximately 60% of participants having a support person present is indicative of successful implementation (Misri et al., 2000).

Logic Model Description

As shown in **Figure 1: Burke's Birth & Wellness Mobile Clinic Logic Model,** the program focuses on enhancing maternal and infant health. Inputs include a range of shareholders that include community doulas, health care providers, and a clinic director. Material inputs include resources like grant funding, curriculum materials, and a mobile clinic van. Key activities such as recruiting community members to become trained doulas, increasing access to pre- and post-natal appointments, and providing group health education classes. Outputs will be measured through metrics such as the number of recruited doulas, the number of perinatal care appointments attended and offered, and the number of educational classes provided each month.

The outcomes within our logic model encompass short-term, medium-term, and long-term goals. These include increased knowledge about the benefits of perinatal care among community members, greater availability of doulas and healthcare providers in rural areas, and ultimately, a reduction in both infant and maternal morbidity and mortality rates. The implementation of the program is expected to substantially improve the quality of life for mothers and infants in rural areas by fostering better health outcomes and support systems.

Inputs

Shareholders for the program include community doulas who will provide direct support to participants, a clinical provider (OB-GYN, NP, or physicians assistants (PA)) who will offer medical care to program participants, and a clinic director who will oversee the workflow. The resources available to the program, such as curriculum worksheets and a mobile clinic van that will facilitate access to care in rural health care settings, would be provided using grant funding and donations. The input section also includes a curriculum for educational sessions with the goal of educating pregnant people and their support persons on health behaviors while pregnant. Training for doulas will be made available through a third-party program run by Healthy Mothers, Healthy Babies Coalition of Georgia. Community outreach will increase community engagement in healthy pregnancy behaviors and access to care.

Activities

The program uses several key activities designed to promote maternal and infant health. The first major activity is recruiting and training doulas through a third-party program. This activity aims to establish a well-trained network of doulas who will provide support to birthing people. Our program will recruit local candidates to encourage community engagement.

Secondly, our program will offer pre-natal and post-natal appointments. Appointments with providers will ensure consistent monitoring and care for mothers. These appointments will be given monthly during pregnancy and twice postpartum to each involved pregnant individual throughout the duration of the program. In addition to appointments with providers, doula appointments will be offered monthly. Family-centered educational classes will be held on the first Saturday of each month. These classes will engage social networks of birthing people in discussions about perinatal care and healthy behaviors surrounding pregnancy and postpartum.

Outputs

The logic model outlines outputs that will help to demonstrate the program's success. The number of doulas recruited and trained will be measured. Additionally, provider and doula appointments will be measured. Provider appointments are measured throughout the perinatal period. Doula touchpoints include education, material support, and unscheduled visits.

One educational class will be provided every month across 12 months. This ensures that the program is engaging in the community and encouraging social network support for each pregnant individual. These outputs will help evaluate the program's reach and effectiveness in engaging the community in maternal health initiatives.

Outcomes

<u>Short-Term Outcomes</u>

An increase in trained doulas in the community is expected. Participants will demonstrate improved knowledge of healthy maternal care behaviors. Community members will adopt healthier maternal health practices and engage more frequently with available healthcare services due to the education given through this program.

Medium-Term Outcomes

Doula and provider availability will increase as more doulas ae trained in the community, and more providers will be coming to the area to provide care in our mobile clinic van. Healthy behaviors in pregnancy will be practiced by participants as well as the community at large. A stronger support network will develop, with increased engagement in perinatal care initiatives.

Long-Term Outcomes

Over time, there will be lower rates of infant health complications and deaths. The initiative aims for a significant decline in maternal health issues, drastically improving the overall health outcomes for mothers in the community.

The anticipated improvements in program participants' lives include enhanced maternal well-being, better health outcomes for infants, and increased social support. The outcomes from the program's activities are expected to foster healthier families, improve social support systems, and educate the community on healthy practices.

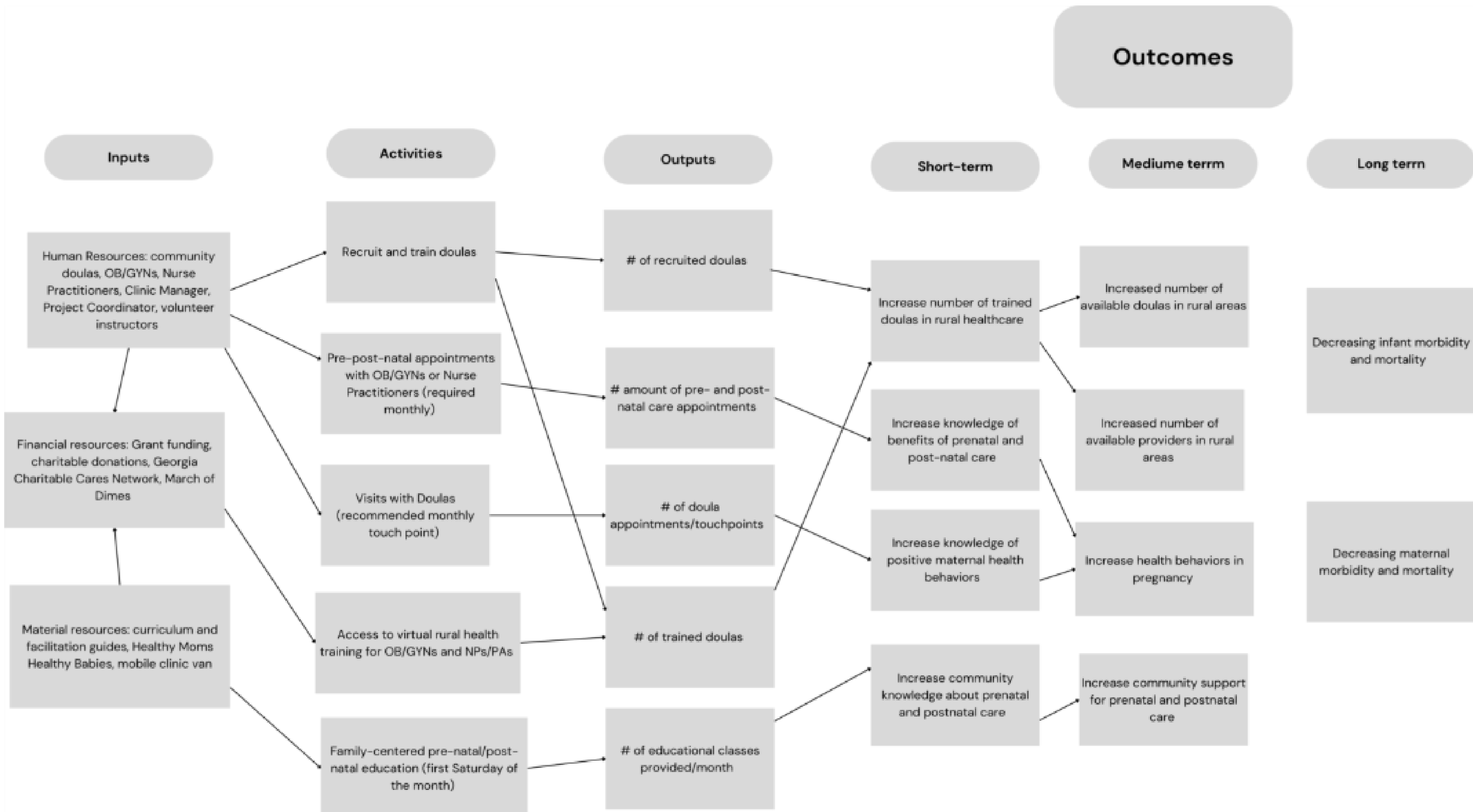

Figure 1. Burke's Birth & Wellness Mobile Clinic Logic Model

Program Description and Intervention Strategies

The proposed program has three main components: training staff, individual care, and group health education. The training staff component will take place prior to the start of the program, while the individual care and group health education will occur once the program sees patients. All aspects of the program will be provided at no cost to the patient.

The program strategies and methods were drawn from three main theories: Social Planning Theory, Social Cognitive Theory, and Theories of Social Network and Social Support. Social planning theory drove the big picture decisions involved in the development of the program, such as use of community-based doulas and evidence-based health education for our group classes. The theory emphasizes fairness and equity, community involvement and participation in decision making, being future oriented and sustainable, and being evidence driven (Rothman J., 2007).

The Social Cognitive Theory underpins many of the methods utilized for health education and doula sessions. This theory emphasizes that learning is shaped by the interaction between behavior, personal factors, and the environment. It also emphasizes learning through observation and highlights the importance of self-efficacy in learning and motivation (Bandura, A., 1986). The constructs of active learning and guided practice from this theory will be directly applied during doula sessions and group health education to reinforce learning and promote self-efficacy of positive maternal health behaviors (Bartholomew Eldredge et al., 2016).

The program also draws on Social Network and Social Support theories (Bartholomew Eldredge et al., 2016), which highlights the power of building or strengthening social connections to sustain behavior change. By educating and engaging these networks, individuals receive ongoing reinforcement from people they trust. This principle is used throughout our approach, through the integration of community-based doulas and designated support persons. This approach leverages existing relationships or forms new, trusted ones to provide consistent health education, encouragement for behavior change, and collective community transformation.

The strategies utilized to support the program goals and objectives will be described below, for more detailed information see **Appendix B.**

Training Staff

Prior to the beginning of the program, two part-time doulas from the community will be recruited. Their roles will be described in detail in later paragraphs. They will be individuals from Bourke County or nearby counties. They will participate in the developed training from Healthy Mothers, Healthy Babies Coalition of Georgia. The Coalition offers a free scholarship for low-income individuals who are interested and committed to becoming doulas (Healthy Mothers, Healthy Babies Coalition of Georgia, 2025).

In addition, a part-time rural health care provider will be recruited for clinical care. This may be an OB-GYN, NP, or PA, depending on availability and clinical experience. This provider will be offered one week of online training in maternal care disparities and rural health from NetCE continuing education (https://www.netce.com/). Literature shows that participation in rural health education can help improve provider confidence and preparation (Eddy et al., 2025).

These training programs aim to address the determinant of lack of healthcare provider knowledge about rural health by capitalizing on the strategies of technical assistance, as well as use of lay health workers and peer education from the Models of Community Organization (Bartholomew Eldredge et al., 2016). Providing training opportunities for staff gives the structural support needed to improve knowledge and skills related to maternal rural health. The collaboration between community doulas and the provider can further improve their knowledge of rural health, as the doula can provide essential context as a member of the community.

Individual Care and Education

Every client in the program will participate in the individual care component of the intervention. This includes care from both the clinical provider and the doula. The clinical provider's services will occur in a mobile van parked in accessible areas like community centers, grocery stores, places of worship, and neighborhood streets (Mobile Health in Baltimore Consensus Conference Report, 2024). Locations will rotate based on the current caseload needs. The van will carry all necessary supplies, including exam equipment, diagnostic tools, medical items, medications, and supporting infrastructure (Weisel, 2024). Essential tools will be chosen in partnership with the clinical provider.

The provider will deliver perinatal care to each birthing person in the program. The suggested schedule is at least one monthly visit during pregnancy and two during the postpartum period, with adjustments for individual medical needs, to match evidence-based clinical practice guidelines (ACOG, 2020; ACOG, 2025). These visits will include prenatal exams, ultrasounds, and relevant health education related to gestational age.

The doula will offer culturally competent health education, plus physical and emotional support, to birthing persons and their support networks throughout the perinatal period (Darvish et al., 2024). They will coordinate with the provider for continuity of care. Doulas will reinforce the provider's education and help birthing persons brainstorm solutions for adopting healthy maternal behaviors. The doulas also will provide essential infant care resources that clients may need, like diapers, formula, and bottles during the first 3 months of care, while setting them up with more long-term solutions for the future through additional community resources. The recommended doula contact is once a month, tailored to personal needs, and sessions can be virtual or in-person home visits as appropriate.

The individual care component will cover key determinants like limited physical access care, limited knowledge about maternal care, perceived benefits and barriers, and self-efficacy in meeting maternal needs. By introducing mobile health vans, healthcare services are provided in areas that are more accessible to the community with flexibility in location. Therefore, the intervention engages in systems change by adapting healthcare delivery to better serve our population, who has difficulty physically accessing services. Using lay health workers like doulas also helps access by expanding the local workforce (Bartholomew Eldredge et al., 2016).

Gaps in knowledge, perceived benefits and barriers, and self-efficacy will be addressed through teamwork among the patient, the clinical provider, and the doula. Each birthing person's care plan, developed in partnership with the clinical provider, will be individualized to their needs. Doulas will stay updated on these plans, enabling them to reinforce education at the participant's pace; something traditional providers may lack time for. Throughout the program, participants will receive feedback on risks from unhealthy behaviors from both doulas and the clinical provider implementing the method of consciousness raising. Building on this collaborative education and feedback, doulas will use their community roots to strengthen participant engagement and problem-solving.

As community members, doulas foster trust through cultural alignment. This bond positions the doula as a supportive guide, using probing questions to help participants identify solutions to barriers. They will then assist patients in creating specific action plans for healthy behaviors, drawing from the methods of coping strategies, and implementation intentions.

Group Health Education

The final component of the model is group health education. Participation is voluntary yet strongly encouraged, and pregnant individuals are asked to invite one support person (e.g., partner, family member, or friend) to attend with them. Sessions will occur for 90 minutes on the first Saturday of each month for 12 months at a centrally located community center or other accessible venue in Burke County. These sessions will occur on Saturdays in anticipation that participants and instructors will have more availability on weekends. Locations may be adjusted based on community feedback. The mobile van will be stationed at the same site on class days, allowing participants to schedule appointments immediately before or after the session.

Each 90-minute class will be co-facilitated by a doula and a volunteer community member who has experience giving birth and providing peer support via formal training or certification. This ensures ongoing community leadership and relevance. There will be two volunteer instructors. The curriculum is adapted from the evidence-based Antenatal Education Resource Guide for Parent Educators developed by Ireland's National Antenatal Education Programme (National Antenatal Education Programme for Women and Their Chosen Birth Partners, n.d.). Specific topics, including nutrition, labor, infant care, and postpartum recovery, will be selected and culturally tailored to the priorities and context of Burke County families.

These classes target inadequate knowledge of both the birthing person and social network, as well as low self-efficacy of the birthing person around taking care of maternal health needs. These classes will incorporate strategies of active learning and guided practice. Learning is hands-on: participants repeatedly practice skills such as safe pregnancy exercise and basic infant-care tasks, receive immediate feedback, and engage in peer and facilitator discussion (Bartholomew Eldredge et al., 2016). The presence and engagement of support persons promote cooperative learning and ultimately enhances network linkages (Bartholomew Eldredge et al., 2016). Strengthening the knowledge of social networks has been shown to improve maternal and infant outcomes (Bartholomew Eldredge et al., 2016; Stapleton et al., 2014)

Work Plan

The work plan includes program objectives, and the activities and subtasks needed to achieve each objective (see **Appendix C**). Each activity or subtask is assigned to the staff member(s) responsible for its execution, along with any necessary resources or data. The plan includes timelines and due dates for each activity or subtask and the main objective. Tasks and dates within each main objective are in chronological order. Estimated timelines for all tasks include a buffer to ensure flexibility for changing priorities and demands.

The plan begins with a partnership between March of Dimes Mom & Baby Health Centers and Burke's Mobile Clinic. Burke's Mobile Clinic will identify service areas, provide clinical services, build partnerships, and manage daily operations. March of Dimes will build the mobile clinic and offer marketing, supervisory, and resource support, as well as stipends for non-clinical services (March of Dimes, 2025). The initial project team at Rollins School of Public Health will also establish a partnership with a regional or state clinical provider and the Healthy Mothers, Healthy Babies Coalition of Georgia (HMHBGA) for additional resource support.

Program personnel include a full-time remote (in Georgia) program director, a part-time maternal healthcare provider, two part-time doulas, and two volunteer instructors. The program director leads all programmatic activities. The maternal healthcare provider leads all clinical activities. The doulas facilitate community-based care by providing both clinical and programmatic support to the program, as well as educational, emotional, and social support to program participants. Doulas also facilitate group health education in support of the volunteer instructors. The program director and doulas are required to have a personal vehicle upon employment and will receive mileage reimbursements. The work plan documents two contract positions for this program: a part-time driver for the mobile clinic and a part-time researcher for program evaluation. See Table 1 for a detailed description of Burke's Mobile Clinic's program implementation plan.

Evaluation Plan

The Burke Mobile Clinic Program will be evaluated using a combination of process and outcome evaluation strategies. Each evaluation strategy corresponds with at least one of our previously mentioned process and outcome objectives. The major outcomes to be evaluated are the uptake of clinical provider and doula visits among birthing people in Burke County, improvements in knowledge of positive maternal health behaviors and resources among program participants, and the attendance of members of birthing program participants' support networks at group maternal health classes.

Process Evaluation

Process evaluation will assess whether the planned components of Burke Mobile Clinic were implemented correctly. The main processes to evaluate are the recruitment of community doulas, the set up of doula visits with program participants, and the development and delivery of group maternal health education classes. The mobile clinic will primarily track the dose received (i.e., attendance) as an overall measure of the effectiveness of delivering our program because literature shows more frequent attendance is a predictor for higher quality participant engagement (Dumas et al., 2006).

Recruitment will be evaluated based on whether we have 2 certified doulas and how easily community members were able to complete their doula training. To obtain this information, the full-time clinic director will conduct key informant interviews with certified doulas at the end of the training period to ascertain what the barriers and facilitators were to completing the training and becoming certified. To then evaluate the process of community doulas having monthly visits with program participants, the clinical director will coordinate with the doulas each month to ensure that both doulas are scheduled to see 4-6 program participants in a month.

To evaluate the effectiveness of delivering the group maternal health classes, the volunteers and community doulas will work together to create a schedule for the first 12 months of program implementation detailing which dates each class will be held and what the topic for each class will be. Doulas will send share this schedule with the program leads so a record can be kept of the number of planned maternal health visits. Volunteer instructors will mark when classes are completed at the conclusion of each course and share with program lead. This will be done monthly for all 12 months of program implementation.

Attendance sheets will be used to track the number of program participants who attend the monthly classes. Participant attendance will be taken upon arrival by the volunteer instructor. During each class, participants will sign in on an attendance sheet and mark a box to indicate a support person has attended. The total number and percentage of birthing persons enrolled as participants who attended the maternal health classes will be recorded at the end of the program period. Describe the “context” process eval part here – note who will conduct the survey and explain that you need an external evaluator to avoid conflict of interest. The process evaluation table can be viewed at **Appendix D**.

Outcome Evaluation

Outcome evaluation will be conducted by a hired program evaluator after the first twelve months of program delivery are completed. The major outcome to be evaluated is whether participants saw an increase in their knowledge of positive maternal health behaviors and where they can acquire resources. This outcome will be measured using a pre-test/post-test design where participants' scores on the post-test will be compared to their scores on the pre-test.

Uptake in the number of clinical provider visits as well as the number of doula visits will be evaluated using the free attendance tracking software MyAttendanceTracker (MyAT). MyAT was primarily designed for teachers to track student attendance but can be scaled for any type of attendance tracking purposes. MyAT will provide metrics on each person whose attendance is tracked which can be sorted and filtered to allow users to track the progress of their attendees such as the number of clinical provider visits attended by a program participant. MyAT can be downloaded as a mobile app onto any mobile device and data stored in the app can be imported and exported (MyAttendanceTracker.com).

Participant self-efficacy in engaging in positive maternal health behaviors and practices will be measured using a modified version of the Childbirth Readiness Scale (CRS) and the Healthy Eating and Weight Self-Efficacy (HEWSE) scale. The CRS was developed by Mengmei et al. (2022) and is an 18-item scale designed to comprehensively assess a birthing individual's preparedness for childbirth, whereas the HEWSE scale is an 11-item scale developed by Wilson-Barlow et al. (2014) to assess an individual's belief in their ability to engage in behaviors that help to maintain a healthy diet and weight. The CRS covers four dimensions related to pregnancy and childbirth: self-management, information literacy, birth confidence, and birth plan (Mengmei et al., 2022). Items consist of statements such as "I take care of my hygiene during pregnancy" and "when necessary, I take medication as prescribed by my doctor during pregnancy" and are measured using a five-point Likert-scale ranging from 1 (strongly disagree) to 5 (strongly agree), with higher scores indicating higher childbirth readiness (Mengmei et al., 2022). The HEWSE scale targets general beliefs about healthy eating and maintaining a healthy weight with items such as “I am able to consume fruits and vegetables in most of my meals” and “I am able to eat a variety of healthy foods to keep my diet balanced” (Wilson-Barlow et al., 2014). Like the CRS, items on the HEWSE scale are scored on a 5-point Likert scale of 1 (strongly disagree) to 5 (strongly agree) with higher scores indicating higher self-efficacy (Wilson-Barlow et al., 2014).

To adapt these scales to our program, the CRS was pared down to 13 items to center on questions related to knowledge and behaviors during pregnancy, as we intend most participants to join the Burke Mobile Clinic program earlier in their pregnancy. Three items from the HEWSE scale were included to capture participants' feelings on their ability to consume healthy foods during pregnancy. The result will be a 16-item scale which uses Likert-scale of 1 to 5with higher scores indicating higher self-efficacy. This scale will be administered in a pre-test/post-test format. Participants will complete this at the start of the first maternal health class they attend and at the end of the last class in the program. Increased participant self-efficacy is the percentage of participants who had at least a 1-point increase in their score between pre-test and post-test.

To evaluate the investment of participants’ social networks in maternal health, we will utilize paper attendance sheets to track the number of program participants who attended the group maternal health education sessions with a support person. Participant attendance will be taken upon arrival by program volunteers. A check box will indicate a support person was present. These attendance sheets will be collected at the start of each monthly health education session for the first 12 months of program implementation (a total of 12 sessions). Following the conclusion of the final session, we will add up the total number of participants who attended across all sessions, and then we will calculate the percentage of program participants who had a support person attend during at least 50% of educational sessions. The outcome evaluation table can be viewed at

Appendix E.

Data Analysis

The adapted CRS/HEWSE pre-test and post-test will be administered in Microsoft forms and on paper for those who have trouble accessing the internet. Scores, data from attendance sheets, and data recorded in MyAT will be tracked in an Excel spreadsheet. The MyAT attendance spreadsheet will be uploaded into SAS, and simple descriptive statistics will be calculated to show the average number of provider visits and doula visits, as well as the mean pre-test and post-test scores after the first 12 months of implementation. In Microsoft Excel, program staff will calculate the frequency and percentage of participant attendance and social support person attendance, respectively.

Limitations

Our maternal health classes are voluntary, risking measurement bias due to inconsistent attendance for pre- and post-test collection. To mitigate this, we will administer the pre-test at each participant's first attended session (even if not the program's first), noting the attendance date. For participants missing the final class, we will provide the post-test at their next provider visit.

Program Timeline

The program timeline (**Appendix F**) provides a two-year overview of the objectives, activities, and subtasks outlined in the work plan. The timeline is in a Gantt chart format to organize tasks by objectives and display where tasks begin, end, and run concurrently. Mirroring the work plan, tasks are broken into the following sections: 1) Secure Program Partners, 2) Hire Program Personnel, 3) Program Staff Training, 4) Acquire and Store Material Resources, 5) Program Delivery - Individual Care and Education, 6) Program Delivery - Group Health Education, and 7) Program Evaluation.

Budget Justification

The total cost for program implementation is $381,096, with $158,269 budgeted for year one and $222,827 for year two (see **Appendix G**). The budget accounts for one full-time program director to ensure that this lead role has enough capacity dedicated to program implementation. The maternal healthcare provider and doulas are all part-time positions (50% FTE). According to the American Community Survey (ACS) 2023 5-year estimates, 4.6% (256 total) of women in Burke County between the ages of 15 and 50 gave birth in 2023 (US Census Bureau, 2023). Based on this estimation, one part-time clinical lead and two clinical support roles are sufficient for program implementation. The overall personnel cost is $297,917.

The mobile clinic vehicle costs $0 because the cost will be absorbed by the March of Dimes Mom & Baby Health Centers program. Additional transportation costs include gasoline for the mobile clinic and mileage reimbursements at $0.67 per mile for the personal vehicles operated by the program director and doulas. In total, transportation costs amount to $6,500. The program budget accounts for office and clinical equipment and supplies, storage and event space rentals, as well as maternal and infant care supplies. Some equipment and supplies have a $0 cost because the program director will source maternal care clinical supplies, infant care supplies, and supplies for prenatal and infant classes from the clinical provider and other regional, state, and national maternal health providers. A small budget of $1,000 is reserved in year two to account for potential supply shortages. The total cost of equipment and supplies is $8,900. The program budget also accounts for the cost of creating, adapting, and printing educational and promotional materials for the clinic setting, group health classes, and the program more broadly. These materials are estimated to cost $1,734.

Miscellaneous costs amount to $1,400. They are associated with the group health education component of the program, including the costs of technology rentals, food, and beverages. Volunteer instructors are unpaid; however, the budget accounts for a $250 honorarium in acknowledgement of their efforts. Lastly, purchased services account for costs paid to third-party contractors, including a mobile clinic driver (part-time, 50%) and a researcher (part-time, 50%) for program evaluation. These contract positions will cost a total of $27,900. Purchased services also include $2,000 worth of technical and vehicle support at $50 per hour. Personnel training costs are also accounted for. Doula trainings have no associated cost as part of the Building Perinatal Support Professionals (BPSP) program facilitated by Healthy Mothers, Heathy Babies Coalition of Georgia (Healthy Mothers, Healthy Babies, 2022). The NetCE training for the Healthcare provider costs $100.

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Appendices

Appendix A. PRECEDE Model of Burke's Mobile Clinic

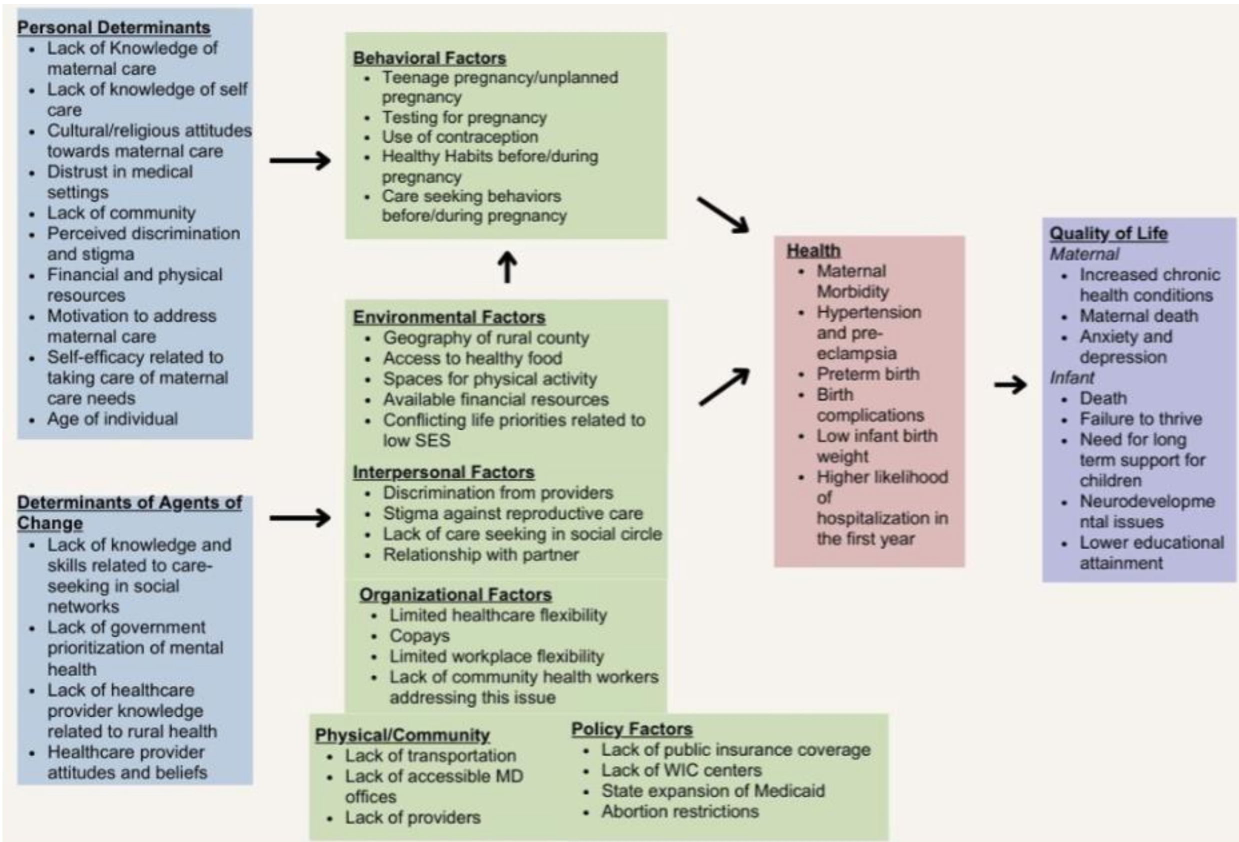

Figure 2. PRECEDE Model of Burke's Mobile Clinic

Appendix B. Key Determinants and Program Strategies

Table 2. Key Determinants and Strategies Used by Burke's Mobile Clinic

Key Determinants	Theory-Driven Intervention Strategies	Program Application
***Individual:* Lack of knowledge of maternal care**	Active learning (Social Cognitive Theory) Use of lay health workers & peer education (Theories of Social Network and Social Support; Models of Community Organization) Individualization (Transtheoretical Model) Mobilizing social networks (Theories of Social Network and Social Support)	**Active learning & Use of lay health workers & peer education:** Birthing individuals will engage in active learning by attending perinatal care appointments and actively participating in perinatal care education classes. Educational classes will include activities that engage the participant **Individualization:** Each woman will be assigned to a community-based doula allowing them to learn at their own pace **Mobilizing social support:** Doulas will use prompting communication techniques to provide social and emotional support to birthing individuals
***Individual:* Perceived benefits of and barriers to maternal care**	Consciousness raising (Health Belief Model; Transtheoretical Model) Planning Coping Responses (Theories of Goal Directed Behavior) Cultural Similarity (Communication Persuasion Network)	**Consciousness raising:** Both the clinical providers and community doulas will provide feedback and education on potential consequences of unhealthy behaviors **Planning Coping Responses:** Doula and the birthing person have discussions regarding perceived barriers to behavior and use cuing techniques to help identify solutions **Cultural Similarity:** Doulas will come directly from the Burke County community (or surrounding areas if more staff needed) and will likely be more culturally similar to participants
***Individual:* Self-efficacy related to taking care of maternal needs**	Implementation Intentions (Theories of Goal Directed Behaviors) Guided Practice (Social Cognitive Theory)	**Implementation Intention:** In collaboration with the mobile care providers (both practitioners and doulas), birthing individuals will be prompted to make specific plans on how they will carry out healthy behaviors **Guided Practice:** Education for birthing persons will start with their provider, and doulas will offer reinforcement. Discussion in classes will offer

		anecdotal and experiential options and viewpoints from peers
***Environmental:* Lack of physical access to maternal care providers**	Systems change (Systems Theory) Use of lay health workers; Peer education (Theories of Social Network and Social Support; Models of Community Organization)	**Systems change:** The use of mobile clinics will combat this lack of access due to increased availability of providers for pregnant people **Use of lay health workers & peer education:** The use of doulas as an allied health professional will address the lack of maternal health workforce available in these areas
***Interpersonal:* Lack of knowledge and skills related to maternal care in social network**	Mobilizing social networks (Theories of Social Network and Social Support) Cooperative learning (Theories of Stigma and Discrimination) Enhancing network linkages (Theories of Social Network and Social Support)	**Mobilizing social networks:** Mobile care providers will support the network's capacity to provide informational and emotional support to the birthing person **Cooperative learning:** Group classes will create opportunities for network members to provide support and learn alongside the birthing person **Enhancing network linkages:** Knowledgeable social networks can increase positive outcomes during and after pregnancy for pregnant individuals.
***Environmental/Interpersonal:* Lack of healthcare provider knowledge related to rural health**	Technical Assistance (Models of Community Organization) Use of Lay health workers; peer education (Theories of Social Network and Social Support; Models of Community Organization)	**Technical assistance:** OB/GYNs will receiving training in rural maternal care at the start of the program **Use of Lay health workers; peer education:** Doulas may have more context about providing care in rural places, which can complement the clinical provider's professional expertise and allow them to gain more insight into working in rural communities

Appendix C. Project Workplan Table

Table 3. Burke's Mobile Clinic Project Workplan

Objective	Activities/Subtasks	Staff Responsible	Resources Needed or Data Sources	Dates	Due Dates
Secure program partners	**March of Dimes:** establish a partnership with March of Dimes Mom & Baby Health Centers **Clinical provider:** Identify regional or state healthcare providers **Healthy Mothers, Healthy Babies Coalition of Georgia:** establish a partnership with HMHBGA Either initiate contact via email or phone correspondence. Use formal or informal networks for a warm hand-off to an institutional representative Detail program plans and identify the specific role and expectations for a clinical provider Finalize contracts with selected clinical provider	Burke's Mobile Clinic planning team at Rollins School of Public Health	Technology and outreach tools: Computer, internet access, phones, Microsoft Office Suite Formal contracts and/or Memorandum of Understanding (MOUs)	January 5, 2026, to December 31, 2027	Official contract signed by January 30, 2026
Hire program personnel	**One part-time rural health care provider (Aug 2026 – Sep 2026):** **one full-time clinic director (remote in Georgia), (Mar 2026 – Dec 2027):** Create job descriptions	Burke's Mobile Clinic planning team at Rollins School of Public Health Partners: March of Dimes, the Clinical	Technology and outreach tools: Computer, internet access, phones, Microsoft Office Suite LinkedIn, Indeed, and 12Twenty accounts and job postings for each platform	January 5, 2026, to February 27, 2026	Have the three candidates sign employment contracts by February 27th

	Post job openings via LinkedIn, Indeed, Glassdoor, and ZipRecruiter Promote the roles across local networks like March of Dimes, the Clinical partner, word-of-mouth, and HMHBGA Review resumes and cover letters Conduct virtual interviews Offer positions to select candidates Finalize employment once offers are accepted	Partner, and HMHBGA			
Hire program personnel	**Two part-time Burke County residents as program Doulas:** Create job description along with physical and virtual promotional material specific to the Doula roles Use promotional material on social media and local job boards. Distribute flyers at community centers, clinics, and other spaces where community members gather or visit. Identify two Burke County residents via recruitment efforts Review resumes and cover letters Conduct virtual interviews	Burke's Mobile Clinic planning team at Rollins School of Public Health	Technology and outreach tools: Computer, internet access, phones, Microsoft Office Suite Physical flyers and informative printouts Table and chairs for on-site promotion	January 5, 2026, to February 27, 2026	Have two candidates sign employment contracts by February 27th

	Offer positions to select candidates Finalize employment once offers are accepted				
Program staff training	**Health Care Provider:** Complete NetCE's online training in maternal care disparities and rural health **Doulas:** complete 6 months of training via the Building Perinatal Support Professionals (BPSP) program facilitated by Health Mothers, Heathy Babies Coalition of Georgia	Clinic Director confirms that the Doulas and Healthcare Provider complete their corresponding trainings	Technology and outreach tools: Computer, internet access, Microsoft Office Suite NetCE Continuing Education platform account Access to the Building Perinatal Support Professionals (BPSP) program	March 16, 2026, to August 28, 2026	Health care providers complete training at any point before May 16th Doulas complete training program by August 28th
Acquire and/or store program resources	Acquire a storage space for program material Acquire a clinic documentation system (Microsoft Office Suite) Recruit a contracted mobile clinic driver Source donations of maternal care clinical supplies, infant care supplies, and supplies for prenatal and infant classes from the clinical provider and other regional, state, and national maternal health providers. Purchase additional maternal care clinical supplies, infant care supplies, and supplies	Clinic Director	Technology and outreach tools: Computer, printer, internet access, and virtual writing, tools, phones, email correspondence Driver contract Personal vehicle for material transport Appropriate tax forms for cash or noncash charitable contributions Storage space for program supplies Dedicated garage or parking space for the mobile clinic vehicle	March 2, 2026, to March 31, 2026 April 1, 2026, to July 31, 2026	Storage space and clinic driver contract signed by March 31st Microsoft license purchased by March 31st 80% or program material collected, recorded, and organized in dedicated program spaces by August 28th

	for prenatal and infant classes Source general educational informational materials (like flyers, posters, pamphlets, and infographics) from the clinical provider and other maternal care research and education institutions. Create an inventory record for all program material and a usage log for routine tracking of program material. Use list and log to initiate replenishment as needed			April 1, 2026, to August 31, 2026	All inventory located in storage with a detailed record of the storage contents by August 31st
Deliver program - Individual Care and Education	**Care:** Initial patient contact for program onboarding. Share program details and protocols Execute initial patient-provider prenatal appointments. Create profiles for each birthing person with specific details about their medical and non-medical needs. Routinely coordinate appointment scheduling including reminders and preparation details. Execute routine (at least once a month) pre- and post-natal appointments with program participants in the mobile clinic Follow up between appointments with supplies,	Clinic director Healthcare provider and Doulas Doulas Healthcare provider and Doulas Doulas	Technology and outreach tools: Computer, internet access, phones, Microsoft Office Suite Clinical supplies: mobile clinic, tools, machines, and personal protection equipment, documentation system (Microsoft Office Suite) Educational material Communication tools and technology: phone (with text capability), email, and scheduling platform. Maternal care supplies, instruction/demonstration supplies Personal vehicle for most home visits	September 14, 2026, to September 30, 2027	Monthly contact for patient care is strongly encouraged but not linked to specific due dates. Appointments will be scheduled during each care visit or recurring according to patient preference Appointment confirmations are due a week and a day before each appointment. Doula led follow ups/contact are not required or linked to specific due dates but follow up for a specific issue, demo request, or materials request should be

	informational support, social and emotional support at participant homes or accessible areas like community centers, grocery stores, places of worship, and neighborhood streets **Education:** Provider – Patient education: Deliver comprehensive maternal care lessons during visits. Lessons will be tailored to the patient's gestational progress, and any issues, topics, and questions raised during their appointments. Doula – Patient education: Deliver additional information on topics initiated by the provider, answers patient questions, offers demonstrations, and offers additional information on topics like meals/nutrition, postpartum healing, or newborn care. Doula contact will be by need and at least on a monthly basis.	Healthcare Provider with Doula support Doulas			initiated within a week of the initial request. Monthly contact for educational components between provider and patient is required but not linked to specific due dates. Monthly contact with Doulas for educational, material, social, and emotional support is not required or linked to a specific due date.
Deliver program – Group Health Education	**Group health education:** Adapt the Antenatal Education Resource Guide for Parent Educators curriculum into increments to be	Clinic Director and Healthcare Provider	Computer, printer, internet access, and virtual writing tools The Antenatal Education Resource Guide for Parent Educators curriculum	August 1, 2026, to September 11, 2026 August 1, 2026, to	Group education materials due to class location each Saturday morning

	delivered via group classes Recruit 2 volunteer instructors for prenatal and infant classes Secure physical spaces for classes Adapt and disseminate class promotional material in public community spaces and via social media Deliver 12 prenatal or infant classes for pregnant people and people who have recently given birth covering a range of topics Manage educational materials for group education sessions	Clinic Director Doulas Doulas Volunteer instructors and Doulas Doulas	MOUs Instruction/demonstration supplies, program vehicle, maternal care supplies Physical space for classes Storage space Personal vehicle for material transport	September 30, 2026 Classes every first Saturday of the month October 1, 2026, to September 30, 2027	Signed MOUs for Volunteer Instructors by September 30th
Program evaluation	Create or adapt a short questionnaire for program participants to measure increased knowledge of maternal health behaviors, practices, and resources. Deliver questionnaire at the initial patient – provider appointment and record responses Deliver questionnaire at the sixth patient – provider appointment and record responses Organize and store patient responses Recruit a part-time contractor for program evaluation	Clinical Director	Computer, printer, internet access, and virtual writing tools Childbirth Readiness Scale (CRS) Healthy Eating and Weight Self-Efficacy (HEWSE) scale	June 1, 2026, to July 31, 2026 September 14, 2026, to September 30, 2027 August 27, 2027, to September 2027 October 1, 2027, to	Finalized questionnaire due August 28, 2026 Deliver initial questionnaire to each participant during the initial patient-provider appointment Data collection will be ongoing between September 2026 and September 2027. Post-program questionnaire issued at the sixth month of program participation

	Execute process and outcome evaluation			December 31, 2027	Data analysis, findings, and recommendations due December 31st

Appendix D. Process Evaluation Table

Table 4. Process Evaluation for Burke's Mobile Clinic

	Process-Evaluation Question	Data Sources	Tools/Procedures	Timing of Data Collection	Measure/Indicator
Fidelity	How may community members were trained as doulas?	Program staff files	Receipt of doula certificates from all program staff who completed a doula credentialing program	Certificates will be collected at the end of the 6-month staff training period	Number of trained doulas
Dose Delivered	How many group maternal health classes were developed?	Community doulas and volunteer instructors	Schedule of all planned group maternal health classes	Obtain calendar schedule from doulas as the start of the 12-month program implementation period	Number of group maternal health classes developed
Dose Received	How many group maternal health classes were delivered?	Volunteer instructors and program participants	Documentation of all group maternal health classes that occurred during program implementation (i.e., were not cancelled)	Record the completion of group maternal health classes monthly at the end of each class	Number of group maternal health classes delivered
Reach	How many birthing persons participated in the program?	Community doulas and volunteer instructors	Attendance sheets to keep track of which participants attended each class	Attendance will be taken monthly at the start of each group maternal health class	Number of birthing persons who attended group maternal health classes
Recruitment	What procedures were followed to recruit community members who were interested in completing doula training	Burke Mobile Clinic Program leads	Burke Mobile Clinic leads document all recruitment activities	Document all recruitment activities daily prior to start of 6-month training period	Number of people who signed up to be trained as a doula

Context	What are the barriers and facilitators to completing doula training and certification?	Community doulas	Key Informant Interviews about what barriers and facilitators to completing the training	Key informant interviews administered at the end of the 6-month training period	Reported barriers and facilitators

Appendix E. Outcome Evaluation Table

Table 5. Outcome Evaluation Plan for Burke's Mobile Clinic

Outcome Evaluation Questions	Data Sources/Samples	Data Collection Methods	Measures/Indicators	Timing	Analysis
What percentage of program participants saw a provider at least once while pregnant and twice in the post-partum period (i.e., first 3 months after giving birth)?	MyAT (free attendance tracking service)	Maternal healthcare provider will electronically keep record of each clinic visit they have with program participants	Number of provider visits attended by participants (0 to 12)	Provider visits will be recorded at the end of every month for the first 12 months of program implementation	% of participants who saw a provider at least one visit while pregnancy and 2 post partum
What percentage of program participants saw a doula at least once a month during program duration?	MyAT (free attendance tracking service)	Doulas will electronically keep record of each visit they have with program participants in MyAT	Number of visits provided by doulas (0 to 12+)	Monthly doula visits will be recorded at the end of every month for the first 12 months of program implementation	% of participants who had a monthly doula visit
What percentage of program participants reported increased self-efficacy in engaging in positive maternal health behaviors and	Pre- and post-tests of knowledge of positive maternal health behavior and awareness of how to access maternal health resources	Pre-tests administered at the start of education sessions. Post-tests administered at the end of educational sessions	Modified version of the Childbirth Readiness Scale (CRS) (Mengmei et al., 2022) and the HEWSE scale (Wilson-Barlow et al., 2014)	Pre-tests and surveys will be administered at the start of the first monthly maternal health education session, and post-tests will be administered at the end of the	% of participants who had at least a 1-point increase in score from pre-test to post-test

practices and increased knowledge of resources after receiving the program (is this self efficacy and knowledge? Seems like 2 outcomes				final (12th) session.	
How many program participants had at least one support person attend pre- and post-natal education sessions with them for each visit?	Attendance sheets with the names of program participants with a box to check off if they brought someone with them	Attendance will be taken upon participant arrival prior to the start of each maternal health education session	Number of monthly sessions attended by an individual program participant in which a support person attended with them(0 to 12)	Participant + support person attendance will be collected monthly for the first 12 months of the program	% of participants who had a support person attend an education session with them.

Appendix F. Project Timeline

Table 6. Proposed Project Timeline

Task	Completion Status	Completion Date	Jan-26	Feb-26	Mar-26	Apr-26	May-26	Jun-26	Jul-26	Aug-26	Sep-26	Oct-26	Nov-26	Dec-26	Jan-27	Feb-27	Mar-27	Apr-27	May-27	Jun-27	Jul-27	Aug-27	Sep-27	Oct-27	Nov-27	Dec-27
Secure Program Partners																										
Establish partnership with MoD Mom & Baby Health Centers			■																							
Establish Partnership with a healthcare provider			■																							
Establish Partnership with HMHBGA			■																							
Hire Program Personnel																										
Recruit and hire part-time health care provider			■	■																						
Recruit and hire full-time clinic director			■	■																						
Recruit and hire program Doulas			■	■																						
Program Staff Training																										
Provider: one-week rural health and disparities training										■																
Doulas: Building Perinatal Support Professionals program					■	■	■	■	■	■																
Acquire and Store Material Resources																										
Acquire Microsoft Office Suite License					■																					
Acquire program storage space					■																					
Recruit mobile clinic driver					■																					
Source clinical, infant care, and group class supplies						■	■	■	■																	
Source general educational informational materials						■	■	■	■																	
Create an inventory record and usage log						■	■	■	■	■																
Recruit two volunteer instructors								■	■																	
Adapt patient questionnaire								■	■																	
Program Delivery - Individual Care and Education																										
Patient - Provider monthly contact + education											■	■	■	■	■	■	■	■	■	■	■	■	■			
Birthing person - Doula monthly contact + education											■	■	■	■	■	■	■	■	■	■	■	■	■			
Program Delivery - Group Health Education																										
Adapt curriculum								■	■																	
Group health education												■	■	■	■	■	■	■	■	■	■	■	■			
Program Evaluation																										
Deliver pre-program questionnaire											■	■	■	■	■	■	■	■	■	■	■	■	■			
Deliver post-program questionnaire																		■	■	■	■	■	■			
Recruit part-time contract researcher for program evaluation																						■	■			
Program evauation and recommendations																								■	■	■

Appendix G. Program Budget

Table 7. Program Budget Year 1

Burke County Mobile Clinic							
Budget Period: January 1, 2025 to December 31, 2027							
Project Year:				**YEAR ONE**			
PERSONNEL	Title	Base Salary	% of Effort	Project Salary	Fringe %	Fringe Amount	Total
	Program Director	66,666	100%	66,666	30.0%	20,000	86,666
	Healthcare Provider	38,750	50%	19,375	30.0%	5,813	25,188
	Community Based Doula	12,400	50%	6,200	30.0%	1,860	8,060
	Community Based Doula	12,400	50%	6,200	30.0%	1,860	8,060
	Volunteer instructor	0	50%	0	0.0%	0	0
	Volunteer instructor	0	50%	0	0.0%	0	0
				98,441		29,533	127,974
Fringe Benefit (30%)							
			TOTAL PERSONNEL EXPENSES:				**127,974**
Other Direct Costs:							
TRANSPORTATION COSTS	Mobile Clinic Vehicle					0	**2,585**
	Gas					1,550	
	Mileage reimbursements					1,035	
EQUIPMENT & SUPPLIES	Office Computers (4@$700 ea)					2,800	**4,900**
	Laser Printer					500	
	Microsoft Office Suite Licence ($12.50/user/month)					900	
	Maternal Care Clinical Tools					0	
	Maternal and Infant Care Supplies					0	
	Group Education Space Rental					300	
	Storage Space Rental					400	
	MyAttendanceTracker Software					0	
PROMOTIONAL & EDUCATIONAL MATERIALS	Pamphlets (500 @ $.50 ea)					250	**1,292**
	Posters (300@$2.00 ea)					600	
	Educational handouts (150@ $.50 ea)					75	
	Office supplies (general, paper, ink/toner)					250	
	Canva Subscription (9 mon@$13/mon)					117	
MISCELLANEOUS	AV Equipment Rental					50	**450**
	Food/Snacks/Drinks					400	
			TOTAL OTHER DIRECT COSTS:				**9,227**
Purchased Services:							
Contractor	Part-time Mobile Clinic Driver (14 mons@$18/hr)					5,580	**5,580**
Technical or Vehicle Support	Computer Support, Inc for technical assistance with software (10 hrs@$50/hr)					500	**500**
	Mobile Clinic Vehicle Support (10 hrs@$50/hr)					500	**500**
Personnel Training	Doula Training					0	**0**
	NetCE Training					100	**100**
			TOTAL PURCHASED SERVICES:				**6,680**
Direct Costs:	Personnel + Other Direct Costs + Purchased Services						143,881
Indirect Costs: 10% of Direct Costs							14,388
			GRAND TOTAL YEAR 1:				**$158,269**

Table 8. Program Budget Year 2

Burke County Mobile Clinic							
Budget Period: January 1, 2025 to December 31, 2027							
Project Year:				YEAR TWO			
PERSONNEL	Title	Base Salary	% of Effort	Project Salary	Fringe %	Fringe Amount	Total
	Program Director	60,000	100%	60,000	30.0%	18,000	78,000
	Healthcare Provider	86,250	50%	43,125	30.0%	12,938	56,063
	Community Based Doula	27,600	50%	13,800	30.0%	4,140	17,940
	Community Based Doula	27,600	50%	13,800	30.0%	4,140	17,940
	Volunteer instructor	0	50%	0	0.0%	0	0
	Volunteer instructor	0	50%	0	0.0%	0	0
				130,725		39,218	169,943
Fringe Benefit (30%)							
			TOTAL PERSONNEL EXPENSES:				**169,943**
Other Direct Costs:							
TRANSPORTATION COSTS	Mobile Clinic Vehicle					0	**3,915**
	Gas for Clinic Vehicle					3,450	
	Mileage reimbursements					465	
EQUIPMENT & SUPPLIES	Office Computers (4@$700 ea)					0	**4,000**
	Statistical Software					1,500	
	Laser Printer					0	
	Microsoft Office Suite					0	
	Maternal Care Clinical Tools					0	
	Maternal and Infant Care Supplies					1,000	
	Group Education Rental Space					900	
	Storage Space Rental					600	
	MyAttendanceTracker Software					0	
PROMOTIONAL & EDUCATIONAL MATERIALS	Pamphlets (500 @ $.50 ea)					0	**442**
	Posters (300@$2.00 ea)					0	
	Educational handouts (150@ $.50 ea)					75	
	Office supplies (general, paper, ink/toner)					250	
	Canva Subscription (9 mon@$13/mon)					117	
MISCELLANEOUS	AV Equipment Rental					50	**950**
	Food/Snacks/Drinks					400	
	Volunteer honorarium					500	
			TOTAL OTHER DIRECT COSTS:				**9,307**
Purchased Services:							
Contractor	Part-time Contract Mobile Clinic Driver (14 mons@$18/hr)					12,420	**12,420**
	Part-time Contract Researcher for Program Evaluation (3 mons@$18/hr)					9,900	**9,900**
Technical or Vehicle Support	Computer Support, Inc for technical assistance with software (10 hrs@$50/hr)					500	**500**
	Mobile Clinic Vehicle Support (10 hrs@$50/hr)					500	**500**
			TOTAL PURCHASED SERVICES:				**23,320**
Direct Costs:	Personnel + Other Direct Costs + Purchased Services						202,570
Indirect Costs: 10% of Direct Costs							20,257
			GRAND TOTAL YEAR 2:				**$222,827**

Table 9. Program Budget Combined

Burke County Mobile Clinic							
Budget Period: January 1, 2025 to December 31, 2027							
Project Year:				**BOTH YEARS**			
PERSONNEL	Title	Base Salary	% of Effort	Project Salary	Fringe %	Fringe Amount	Total
	Program Director	126,667	100%	126,667	30.0%	38,000	164,667
	Healthcare Provider	125,000	50%	62,500	30.0%	18,750	81,250
	Community Based Doula	40,000	50%	20,000	30.0%	6,000	26,000
	Community Based Doula	40,000	50%	20,000	30.0%	6,000	26,000
	Volunteer instructor	0	50%	0	0.0%	0	0
	Volunteer instructor	0	50%	0	0.0%	0	0
				229,167		68,750	297,917
Fringe Benefit (30%)							
			TOTAL PERSONNEL EXPENSES:				**297,917**
Other Direct Costs:							
TRANSPORTATION COSTS	Mobile Clinic Vehicle					0	**6,500**
	Gas for Clinic Vehicle					5,000	
	Mileage reimbursements					1,500	
EQUIPMENT & SUPPLIES	Office Computers (4@$700 ea)					2,800	**8,900**
	Statistical Software					1,500	
	Laser Printer					500	
	Microsoft Office Suite					900	
	Maternal Care Clinical Tools					0	
	Maternal and Infant Care Supplies					1,000	
	Group Education Rental Space					1,200	
	Storage Space Rental					1,000	
	MyAttendanceTracker Software					0	
PROMOTIONAL & EDUCATIONAL MATERIALS	Pamphlets (500 @ $.50 ea)					250	**1,734**
	Posters (300@$2.00 ea)					600	
	Educational handouts (150@ $.50 ea)					150	
	Office supplies (general, paper, ink/toner)					500	
	Canva Subscription (9 mon@$13/mon)					234	
MISCELLANEOUS	AV Equipment Rental					100	**1,400**
	Food/Snacks/Drinks					800	
	Volunteer honorarium					500	
			TOTAL OTHER DIRECT COSTS:				**18,534**
Purchased Services:							
Contractor	Part-time Contract Mobile Clinic Driver (14 mons@$18/hr)					18,000	**18,000**
	Part-time Contract Researcher for Program Evaluation (3 mons@$18/hr)					9,900	**9,900**
Technical or Vehicle Support	Computer Support, Inc for technical assistance with software (20 hrs@$50/hr)					1,000	**1,000**
	Mobile Clinic Vehicle Support (20 hrs@$50/hr)					1,000	**1,000**
Personnel Training	Doula Training					0	**0**
	NetCE Training					100	**100**
			TOTAL PURCHASED SERVICES:				**30,000**
Direct Costs:	Personnel + Other Direct Costs + Purchased Services						346,451
Indirect Costs: 10% of Direct Costs							34,645
			GRAND TOTAL YEAR 1 & 2:				**$381,096**